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ATLAS

A clearer picture.

FACIAL SKINCARE

9 REFERENCE SHEETS

3 CHAPTERS / 3 COMPANION WORKSHEETS

Prepared 9 November 2026 / General education, not medical advice

Your reference index

Use these sheets to explore a topic and prepare questions for your clinician. They do not diagnose conditions, establish suitability or provide procedural instructions.

01 / Understanding your skin

001 Your skin barrier

002 Pigmentation explained

003 Skin analysis

02 / Everyday skincare

004 Sun protection

005 Hydrating serums

006 Choosing a moisturiser

03 / Skin treatments

007 RF Needling

008 Chemical peels

009 IPL & laser facials

COMPANION WORKSHEETS

1. Your consultation companion
2. Compare your options
3. Your follow-up notebook

Edition notes and source context appear on the final page.

Your skin barrier

A starting point for understanding everyday skin comfort.



Natural cheek texture in close-up.

01 / What it is

The outer skin layer helps limit water loss and acts as a boundary between the body and its environment. Skin comfort is influenced by products, weather and underlying skin conditions.

02 / How to understand it

Think of the surface as an organised layer rather than an empty canvas. Changes in dryness, stinging or redness can have several causes; a picture alone cannot identify the cause.

03 / What may be discussed

A consultation may explore cleansing habits, moisturisers, sun exposure and the timing of symptoms. Persistent irritation may need medical assessment before a cosmetic treatment is considered.

04 / Before you decide

Write down products used, when symptoms started and any previous reactions. Introduce this history at your appointment rather than relying on a skin-type label.

05 / Questions to ask

Could this be irritation or an underlying skin condition? Which products should I discuss? Should an elective procedure wait until my skin is settled?

06 / Precautions & perspective

This sheet explains a concept, not a treatment routine. Seek medical advice for persistent or worsening symptoms.

SOURCES AND CONTEXT

Harley Street Aesthetics

NHS: sunscreen and sun safety

Clinic links identify business topics, not independent proof of benefit. Other links offer selected further reading. Sources do not endorse this Atlas. Individual clinical assessment is required.

Pigmentation explained

Similar-looking marks can have different causes.



Flat areas of uneven pigmentation on the cheek.

01 / What it is

Pigmentation describes differences in skin colour. A dark area can relate to several processes, so a single label such as dark spot is not a diagnosis.

02 / How to understand it

Colour, distribution, onset and previous inflammation all help a clinician build a history. Similar appearances may require different approaches, especially across different skin tones.

03 / What may be discussed

Harley Street lists pigmentation consultations and several treatment pathways. A clinician may discuss skin care, sun protection or a procedure after assessing the nature of the concern.

04 / Before you decide

Bring a timeline, relevant medicines and details of earlier peels, lasers or reactions. Ask whether the mark needs diagnostic assessment before any cosmetic treatment.

05 / Questions to ask

What type of pigmentation do you suspect? What are the alternatives? How could treatment affect my skin tone? What happens if pigment returns?

06 / Precautions & perspective

New or changing individual lesions need assessment. No treatment can promise permanently even skin. Irritation and procedures can themselves lead to colour changes.

SOURCES AND CONTEXT

Harley Street Aesthetics

Harley Street Medics

NHS: sunscreen and sun safety

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Skin analysis

A conversation supported by imaging, not replaced by it.



A clinician examining facial skin under magnification.

01 / What it is

Skin analysis combines discussion and examination; some pathways also use photographs or imaging tools to document visible concerns.

02 / How to understand it

A scan can help organise a conversation about texture, tone or appearance. Lighting, image quality and the tool used affect what is shown.

03 / What may be discussed

Harley Street Aesthetics offers a skin-analysis pathway. Its outputs should be treated as prompts for a practitioner-led review rather than as a medical diagnosis.

04 / Before you decide

Check how photographs are stored and used. Describe symptoms and history even when they do not show clearly in an image.

05 / Questions to ask

Who reviews the result? What can this tool assess reliably? Can I decline photography? Does the result change the options we discuss?

06 / Precautions & perspective

Automated analysis cannot rule out skin cancer or explain every skin change. Do not use a reassuring score to postpone medical assessment of a concerning lesion.

SOURCES AND CONTEXT

Harley Street Aesthetics

Harley Street Medics

Clinic links identify business topics, not independent proof of benefit. Other links offer selected further reading. Sources do not endorse this Atlas. Individual clinical assessment is required.

Sun protection

Protection is a combination of habits and products.



Sunscreen being spread across the forearm.

01 / What it is

Sun protection aims to reduce exposure to ultraviolet radiation. Shade, protective clothing and sunscreen work together.

02 / How to understand it

SPF and UVA protection describe different aspects of a sunscreen. Neither number means you can remain in the sun indefinitely.

03 / What may be discussed

The NHS advises sunscreen of at least SPF 30 with strong UVA protection, alongside shade and clothing. Follow product instructions and reapply as directed.

04 / Before you decide

Check expiry dates and whether a product suits your skin. Ask your clinician about additional protection around a procedure or a medicine that increases sun sensitivity.

05 / Questions to ask

Does my medicine affect sun sensitivity? When can I use sunscreen after my procedure? What protection is appropriate for my usual activities?

06 / Precautions & perspective

Sunscreen does not block all UV exposure. Avoid burning and do not use tanning as a measure of skin health. Personal advice may differ after a procedure.

SOURCES AND CONTEXT

NHS: sunscreen and sun safety

Clinic links identify business topics, not independent proof of benefit. Other links offer selected further reading. Sources do not endorse this Atlas. Individual clinical assessment is required.

Hydrating serums

What a humectant can do.



A topical serum and glass dropper.

01 / What it is

A serum describes a product format. The word alone tells you little about the ingredients or their purpose.

02 / How to understand it

Humectants, such as glycerin and hyaluronic acid, attract water. A serum may contain them alongside other ingredients.

03 / What may be discussed

Look at the full formula and how it feels on your skin. A simple moisturiser may already contain the same humectants.

04 / Before you decide

Bring the products you use to a consultation. Two different bottles can contain overlapping ingredients.

05 / Questions to ask

What would this add to my routine? Does my moisturiser already meet that need?

06 / Precautions & perspective

A topical serum is different from an injectable treatment. A product that repeatedly stings or irritates deserves reassessment.

SOURCES AND CONTEXT

American Academy of Dermatology: choosing a moisturiser

Clinic links identify business topics, not independent proof of benefit. Other links offer selected further reading. Sources do not endorse this Atlas. Individual clinical assessment is required.

Choosing a moisturiser

A closer look at lotions, creams and ointments.



Cream moisturiser in an open pot.

01 / What it is

Moisturisers help skin retain water. Lotions, creams and ointments differ in texture and how they feel after application.

02 / How to understand it

Humectants attract water, emollients smooth the surface and occlusives help limit moisture loss. One product can combine these functions.

03 / What may be discussed

A lighter lotion may feel comfortable on oily areas. A richer cream or ointment may suit drier areas. Individual preferences matter.

04 / Before you decide

Note which textures feel comfortable and which products cause stinging. Fragrance-free options may be helpful for sensitive skin.

05 / Questions to ask

Is a different texture worth trying? Which ingredients overlap with products I already use?

06 / Precautions & perspective

Persistent cracking, itch or irritation may need medical assessment. A moisturiser alone cannot explain the cause.

SOURCES AND CONTEXT

American Academy of Dermatology: choosing a moisturiser

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RF Needling

How fine needles deliver radiofrequency energy.



A representative RF handpiece and needle cartridge; device designs vary.

01 / What it is

RF Needling uses an array of fine needles to deliver radiofrequency energy beneath the skin surface. The energy produces local heating.

02 / How to understand it

The cartridge, treatment area and device settings affect where heat is delivered. Device selection and use require a trained clinician.

03 / What may be discussed

The clinic may discuss selected texture or skin-quality concerns. The device, treatment area and practitioner's assessment determine the proposed plan.

04 / Before you decide

Discuss skin tone, scarring history, previous procedures and medicines. Ask how the practitioner assesses suitability and manages unwanted pigmentation or burns.

05 / Questions to ask

Why this device? What evidence supports my indication? How many sessions are proposed? What is the recovery plan and total cost?

06 / Precautions & perspective

Serious complications reported with some uses include burns, scarring, fat loss and nerve injury. Ask about the risks for your skin and the proposed device. This is not a home treatment.

SOURCES AND CONTEXT

Harley Street Aesthetics

NHS: considering cosmetic procedures

FDA: radiofrequency needling safety communication

Clinic links identify business topics, not independent proof of benefit. Other links offer selected further reading. Sources do not endorse this Atlas. Individual clinical assessment is required.

Chemical peels

The formulation and assessment matter more than the name.



A peel brush and preparation bowl.

01 / What it is

A chemical peel uses a controlled chemical treatment to affect skin layers. Products and peel depths vary; the word peel alone does not describe their intensity.

02 / How to understand it

Depth, formulation, application and individual skin response influence the effect. A cosmetic acid product and a professional peel are not interchangeable.

03 / What may be discussed

Harley Street lists chemical peels and named peel systems. A consultation should explain the exact formulation and why it is being considered.

04 / Before you decide

Share previous pigment changes, sensitive skin, medicines and earlier resurfacing. Get written preparation and aftercare instructions for the actual product.

05 / Questions to ask

What peel and depth are you proposing? Is it suitable for my skin tone? What downtime should I allow? What happens if healing is delayed?

06 / Precautions & perspective

Peels can cause irritation, burns, colour changes and scarring. This sheet is not a recipe or a set of instructions for performing a peel at home.

SOURCES AND CONTEXT

Harley Street Aesthetics

NHS: considering cosmetic procedures

Clinic links identify business topics, not independent proof of benefit. Other links offer selected further reading. Sources do not endorse this Atlas. Individual clinical assessment is required.

IPL & laser facials

Light-based procedures are not all the same.



An IPL handpiece and protective eyewear. A laser uses a different light source.

01 / What it is

Harley Street lists IPL facials and carbon laser facials. IPL uses a range of light wavelengths; a laser has a different light-delivery principle.

02 / How to understand it

A device's target and the way energy interacts with tissue shape its intended use. A marketing name is not enough to compare two procedures.

03 / What may be discussed

A practitioner may discuss certain pigment, vascular or skin-appearance concerns after assessment. Not every mark is suitable for a light-based treatment.

04 / Before you decide

Discuss your skin tone, recent tanning, medicines and previous reactions. Ask about eye protection and whether a test area is appropriate.

05 / Questions to ask

What is the exact device? What is the intended target? How do you reduce burn and pigment risks? What is the alternative if I am unsuitable?

06 / Precautions & perspective

Burns, pigment changes, scarring and eye injury are important risks to discuss. A new or changing lesion needs assessment before cosmetic light treatment.

SOURCES AND CONTEXT

Harley Street Aesthetics

NHS: considering cosmetic procedures

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Your consultation companion

Turn a concern into a clearer conversation.



01 / My priorities

Write the concern in your own words. What would a useful improvement look like? What would you prefer to avoid?

02 / My history

List previous treatments, medicines, allergies, relevant conditions and any previous reactions. Keep this personal copy private.

03 / My questions

Ask about diagnosis, options including no treatment, expected benefit, uncertainty, downtime, risks and total costs.

04 / My decision space

Record what you understood and what still needs explaining. You can ask for time before deciding.

05 / My next step

Note who will contact you, any agreed investigations and when to review. An appointment is not a commitment to treatment.

Compare your options

A printable framework for an informed choice.



01 / Option A / Option B / No treatment

For each option, note its purpose and why it is being considered. Include waiting or monitoring where appropriate.

02 / Benefit and evidence

Write down the outcome discussed, how likely it is, how long it may last and which evidence supports it.

03 / Risks and recovery

Record common effects, serious risks, time away from usual activities and any care responsibilities.

04 / Time and cost

Include sessions, maintenance, follow-up, travel and any extra charges. Ask what is included in the quoted price.

05 / My choice

What fits your priorities? What information is missing? Discuss this worksheet with the treating clinician.

Your follow-up notebook

Keep the plan, instructions and contact details together.



01 / My agreed plan

Record the treatment or investigation, date and clinician. Use the written instructions from your own treating team.

02 / My instructions

Attach the actual preparation and aftercare information supplied. This notebook does not replace procedure-specific instructions.

03 / My observations

Note the date and changes you want to discuss. Ask the team which changes are expected and which require urgent contact.

04 / My contact details

Write down the clinic's routine contact and urgent out-of-hours plan. Do not wait for a routine review if urgent advice is needed.

05 / My review

Record follow-up dates, questions and any agreed next step. Keep private medical details on your own device or printed copy.

About this edition



Dr. Ahmed

About the Atlas

Nine facial skincare topics, arranged in three chapters. Each sheet includes a photograph, six short sections and links for further reading.

How sources are used

The sheets link to clinic information and selected patient guidance from the NHS, American Academy of Dermatology and US FDA. Clinic links identify topics; they do not independently establish benefit.

What this collection does

The sheets offer general explanations and questions for a consultation. The photographs provide editorial context. No personalised diagnosis, treatment plan, outcome guarantee or prescribing advice is provided.

Using the worksheets

The companion pages are for your own notes. Keep completed copies private. Follow written instructions from your treating clinician; the worksheets do not replace preparation, aftercare or urgent-contact advice.

Business sources

Harley Street Aesthetics: harleystreetaesthetic.co.uk Harley Street Medics: harleystreetmedics.clinic Harley Street Medical Wellness: harleystreetmedicalwellness.co.uk